



Laboratory Investigation Report

Patient Name	: Meera Singh	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 29 Y 5 M 15 D /F	OP/IP No	: OP/131599
Max ID/Mobile	: 459787/8752005200	Collection Date/Time	: 06/Mar/2022 03:23PM
Lab ID	: 1266032200172	Receiving Date	: 06/Mar/2022
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 06/Mar/2022
Passport No.	:		

Immunoassay

TSH,Serum*

Date	06/Mar/2022 03:23PM	Unit	Bio Ref Interval
Thyroid Stimulating Hormone	1.29	µIU/mL	0.34 - 5.6
CLIA			

Interpretation

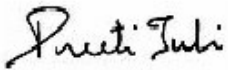
Parameter	Unit	Premature (28 - 36 Weeks)	Cord Blood (> 37 weeks)	Upto 2 Month	Adult	1st Trimester	2nd Trimester	3rd Trimester
TSH	uIU/ml	0.7 - 27.0	2.3 - 13.2	0.5 - 10	0.38 - 5.33	0.1 - 2.5	0.2 - 3.0	0.3 - 3.0

Increased in primary Hypothyroidism.
Decreased in primary Hyperthyroidism

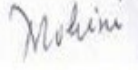
Note : TSH levels are subject to circadian variation, reaching peak levels between 2 – 4 am and at a minimum between 6 – 10 pm. The variation is of the order of 50% - 206 %, hence time of the day has influence on the measured serum TSH concentrations.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Preeti Tuli, M.D.
Sr. Consultant - Lab Medicine



Dr. Mohini Bhargava, MD
Principal consultant (Biochemistry)



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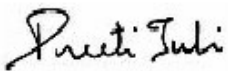
Clinical Biochemistry**Blood Sugar (Random), Fluoride Plasma**

Date	06/Mar/2022 03:21PM	Unit	Bio Ref Interval
Random Glucose UV-Hexokinase	134.2	mg/dl	74 - 140

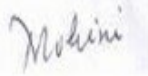
Interpretation A random plasma glucose of > 200 mg/dl with symptoms of diabetes is diagnostic. A confirmatory fasting plasma Glucose (FPG) test or OGTT should be completed on a different day if the clinical condition of the patient permits.

Kindly correlate with clinical findings

*** End Of Report ***



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Clinical Pathology

Urine Routine And Microscopy

Date	06/Mar/2022 03/Dec/21	Unit	Bio Ref Interval
	03:13PM 04:21PM		

Macroscopy

Reflectance photometry

Colour	PALE YELLOW	PALE YELLOW	Pale Yellow
PH	6.5	6.0	5-6
Specific Gravity	1.025	1.015	1.015 - 1.025
Protein	Nil	Nil	Nil
Glucose.	Nil	Nil	Nil
Ketones	Nil	Nil	Nil
Blood	Trace	Nil	Nil
Bilirubin	Nil	Nil	Nil
Urobilinogen	NORMAL	Normal	Normal
Nitrite	Negative	Negative	

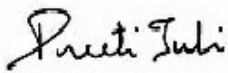
Microscopy

Light Microscopy/Image capture microscopy

Red Blood Cells (RBC)	0-1	Nil	/HPF	Nil
White Blood Cells	0-1	0-1	/HPF	0.0-5.0
Squamous Epithelial Cells	0-1	1-2	/HPF	
Cast	Nil	Nil	/LPF	Nil
Crystals	Nil	Nil	..	Nil

Kindly correlate with clinical findings

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Principal Consultant & Quality Manager
Pathology.



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Hematology

CBC (Complete Blood Count), Whole Blood EDTA

Date	06/Mar/2022 03:13PM	Unit	Bio Ref Interval
Haemoglobin	13.2	g/dl	12.0 - 15.0
Modified cyanmethemoglobin			
Packed Cell, Volume	39.4	%	40-50
Calculated			
Total Leucocyte Count (TLC)	6.7	10~9/L	4.0-10.0
Electrical Impedance			
RBC Count	4.33	10~12/L	3.8-4.8
Electrical Impedance			
MCV	91.0	fL	83-101
Electrical Impedance			
MCH	30.4	pg	27-32
Calculated			
MCHC	33.4	g/dl	31.5-34.5
Calculated			
Platelet Count	269	10~9/L	150-410
Electrical Impedance			
MPV	10.0	fl	7.8-11.2
Calculated			
RDW	14.4	%	11.5-14.5
Calculated			

Differential Cell Count

VCS / Light Microscopy

Neutrophils	57.8	%	40-80
Lymphocytes	35.7	%	20-40
Monocytes	4.9	%	2-10
Eosinophils	1.0	%	1-6
Basophils	0.6	%	0-2

Absolute Leukocyte Count

Calculated from TLC & DLC

Absolute Neutrophil Count	3.87	10~9/L	2.0-7.0
Absolute Lymphocyte Count	2.4	10~9/L	1.0-3.0
Absolute Monocyte Count	0.33	10~9/L	0.2-1.0



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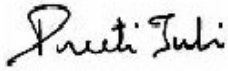
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Hematology

Absolute Eosinophil Count 0.07	10~9/L	0.02-0.5
Absolute Basophil Count 0.04	10~9/L	0.02-0.1

Kindly correlate with clinical findings

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Serology

Test Name	Result	Unit	Bio Ref Interval
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Rapid Card Test - HCV, Serum

Immunochromatography

HCV Card Test Negative

Comment Interpretation

It is only a screening test. All reactive samples should be confirmed by HCV quantitative PCR

A non reactive result does not exclude the possibility of exposure to or infection with HCV.

Patients with auto immune liver disease may show falsely reactive results



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Serology

Test Name	Result	Unit	Bio Ref Interval
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HIV Test (I and II),Serum

CLIA

HIV (I and II)	Negative
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Interpretation

- 1) A Negative result implies that no Anti HIV-1 or HIV-2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been taken during the "WINDOW PERIOD" (before the development of detectable levels of antibodies).
- 2) Positive result suggests the possibility of HIV-1 or HIV-2 infection.

Advise:

To rule out false positivity, false negativity and window period, kindly perform "Confirmatory Tests" like HIV I RNA (Qualitative) Real Time PCR.



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Serology

Test Name	Result	Unit	Bio Ref Interval
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Hepatitis B Surface Antigen, Serum

CLIA

HBsAg	Negative		
HBsAg Test Value	0.42		0 - 0.90

Ref. Range

Negative	< 0.90
Borderline	0.90 - 5.0
Positive	> 5.0

Interpretation

- This test is used to detect hepatitis B surface antigen (HBsAg) in serum sample based on VITROS immunometric immunoassay technique and aid in the laboratory diagnosis of HBV infection.
- Viral hepatitis is a major public health problem with an estimated 257 million persistent carriers of hepatitis B virus (HBV) worldwide. Infection with HBV results in a wide spectrum of acute and chronic liver diseases that may lead to cirrhosis and hepatocellular carcinoma.
- Transmission of HBV occurs by percutaneous exposure to blood products, needle stick injury, sexual contact and perinatally from HBV-infected mothers to baby.
- Hepatitis B surface antigen (HBsAg), derived from the viral envelope, is the first antigen to appear following infection.
- Positive results should be correlated with other potential laboratory abnormalities and clinical picture.
- A negative test result does not exclude the possibility of exposure to or infection with hepatitis B virus.
- Levels of HBsAg may be undetectable both in early infection and late after infection.
- In rare cases HBsAg tests do not detect certain HBV mutant strains.
- HBs Ag disappears with recovery from clinical disease in most patients, however, it persists for years in carriers.



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Test Name	Serology Result	Unit	Bio Ref Interval
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VDRL/RPR, Serum

RPR(Syphilis) Non Reactive
Slide Flocculation

Comment

Interpretation

1. It is a screening test for syphilis which is useful for following the progression of disease and response to therapy. Rising titers are of immense value in confirming the diagnosis.
2. Biological false positive reactions exhibit low titers and are seen in conditions like Viral fevers, Mycoplasma infection, Chlamydia infection, Malaria, Immunizations, Pregnancy, Autoimmune disorders & past history of Treponemal infection.
3. It is advisable to confirm the diagnosis by tests such as TPHA & FTA-ABS.
4. A definitive clinical diagnosis should not be made by result of a single test only, but should be made by taking clinical history and other laboratory findings in to account.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Ranjana Chhabra, M.D.
Senior Consultant Microbiology



Dr. Suchitra Jain, M.D.
Senior Consultant Microbiology



Dr. Saloni Sehgal (MBBS, MD)
Principal Consultant &
Head Microbiology & Infection Control



Dr. Neera Kaushik
Senior Microbiologist



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Hematology

Coagulation Profile

Platelet Count, EDTA

Date	06/Mar/2022 03:21PM	Unit	Bio Ref Interval
Platelet Count Electrical Impedance	269	10~9/L	150-410

Prothrombin Time (with INR), Citrate Plasma

Date	06/Mar/2022 03:21PM	Unit	Bio Ref Interval
Prothrombin Time (PT) Photo-Optical-Nephelometry	10.4	Sec	9.7 - 13.3
MNPT Value	11.9	Sec	
INR	0.87		

Interpretation

(Syn: - Prothrombin Time)

PT is the test which checks the "extrinsic coagulation" pathway and is useful for detecting coagulation deficiency, liver disease and disseminated intravascular Coagulation (DIC).

PT can also be expressed as International normalized ratio (INR) used for monitoring warfarin therapy.

Raised PT value seen in - factor deficiency (Fibrinogen (I), Prothrombin (II), factor V, VII, X), oral anticoagulation therapy, liver diseases, Vitamin K deficiency and DIC.

Advice: - 'PT mixing study', 'specific factor(s) assay' may be added on for further evaluation.



SIN No: B2B1347623, Test Performed at : 969 - Max Hospital, Patparganj, 108A, IP Ext, I.P. Extension, Patparganj, Delhi, 11

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Laboratory Investigation Report

Patient Name	: Meera Singh	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 29 Y 5 M 15 D /F	OP/IP No	: OP/131599
Max ID/Mobile	: 459787/8752005200	Collection Date/Time	: 06/Mar/2022 03:21PM
Lab ID	: 1266032200172	Receiving Date	: 06/Mar/2022
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 06/Mar/2022
Passport No.	:		

Hematology

Coagulation Profile

APTT (Partial Thromboplastin Time), Citrate Plasma, Sodium Citrate

Date	06/Mar/2022 03:21PM	Unit	Bio Ref Interval
APTT	36.6	Sec	25.3 - 34.3
Control	29.8	Sec	

Interpretation

(Activated Partial Thromboplastin Time; PTT)

APTT is the test which checks the "intrinsic coagulation" pathway and is useful for detecting screening of haemophilia A & B, screening of coagulation inhibitors like lupus anticoagulant.

APTT can also be used to monitor heparin monitoring.

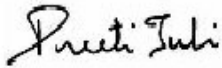
Raised APTT is found in - Liver disease, DIC, heparin treatment, circulating inhibitors (Lupus Anticoagulant), and haemophilia (deficiency of Factor VIII & IX).

Low APTT may be seen in - high concentration Factor VIII and is independent risk factor for increased incidence of thrombus formation.

Advice: - 'APTT mixing study', 'Lupus Anticoagulant test', 'specific Factor(s) assay' may be added on for further evaluation.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Preeti Tuli, M.D.
Principal Consultant & Quality Manager
Pathology.



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